

DISCHARGE SUMMARY

Patient	Meena Pillai	Age/Sex	45 Years / Female
IP No.	SRM-2024-07341	Ward	Endocrinology — Room 214
Admitted	10-Aug-2024	Discharged	14-Aug-2024
Consultant	Dr. Ramesh Iyer, DM Endocrinology	Dept.	Endocrinology & Psychiatry

PRIMARY DIAGNOSIS

- Hypothyroidism (Hashimoto's Thyroiditis) — newly on optimised dose
- Polycystic Ovarian Syndrome (PCOS) — with oligo-amenorrhoea and hirsutism
- Major Depressive Disorder — moderate severity, initiated pharmacotherapy
- Insulin Resistance / Pre-diabetes — HbA1c 6.4%
- Vitamin D Deficiency — severe (14 ng/mL)
- Obesity — BMI 32.4 kg/m², weight 82 kg

REASON FOR ADMISSION

45-year-old female, homemaker, referred from Psychiatry OPD for endocrinology evaluation. Patient presented with persistent fatigue (6+ months), weight gain (8 kg in 1 year), irregular menstrual cycles (last period 3 months ago), excessive hair on chin and upper lip, low mood, difficulty concentrating, and disturbed sleep. TSH found markedly elevated (11.4 mIU/L) on outpatient workup. Admitted for comprehensive endocrine evaluation and initiation of treatment.

SIGNIFICANT HISTORY

- Irregular menstrual cycles since age 28 — previously attributed to stress
- Depression diagnosed 2023 — on Sertraline 50mg initiated by psychiatry
- No prior thyroid disease or treatment before this admission
- Family history: Mother — hypothyroidism. Father — Type 2 Diabetes.

ALLERGY: Codeine — severe nausea, vomiting, excessive drowsiness. AVOID all opioids.

INVESTIGATIONS ON ADMISSION

Test	Result	Reference	Status
TSH	11.4 mIU/L	0.5–4.5	HIGH ↑↑ — significant hypothyroidism
Free T4	0.7 ng/dL	0.8–1.8	LOW — below normal
Free T3	2.2 pg/mL	2.3–4.2	BORDERLINE LOW
Anti-TPO Antibodies	168 IU/mL	< 35	HIGH — Hashimoto's confirmed
LH	14.2 mIU/mL	Follicular 2–15	Elevated relative to FSH
FSH	4.1 mIU/mL	Follicular 3–10	Normal
LH/FSH ratio	3.5	< 2 typical	ELEVATED — PCOS pattern
Total Testosterone	0.71 ng/mL	0.06–0.82	Upper normal
Fasting Blood Sugar	118 mg/dL	70–100	BORDERLINE HIGH

HbA1c	6.4%	< 5.7%	PRE-DIABETIC
Fasting Insulin	22.1 µIU/mL	2–25	Upper normal — insulin resistance
HOMA-IR	6.4	< 2.5	HIGH — insulin resistance
Vitamin D (25-OH)	14 ng/mL	30–100	SEVERELY DEFICIENT
Vitamin B12	398 pg/mL	200–900	Normal
Prolactin	21 ng/mL	2–29	Normal
Pelvic Ultrasound	Multiple follicles bilateral ovaries (>12), enlarged ovaries, PCOS morphology confirmed		

TREATMENT INITIATED DURING ADMISSION

- Levothyroxine 50mcg OD — initiated. Dose to be titrated based on TSH response.
- Metformin 500mg BD — for insulin resistance and PCOS
- Myo-Inositol 2g BD — adjunct for PCOS (OTC supplement)
- Vitamin D3 60,000 IU weekly — for 12 weeks, then reassess
- Calcium + Magnesium 500mg OD
- Sertraline 50mg OD — continued from psychiatry
- Dietary counselling: low-GI diet, calorie deficit, high fibre
- Physiotherapy referral: 150 mins moderate exercise per week

DISCHARGE MEDICATIONS

Medication	Dose	Frequency	Notes
Levothyroxine	50mcg	Once daily (empty stomach)	Take 30 min before food
Metformin	500mg	Twice daily with food	For PCOS + pre-diabetes
Myo-Inositol	2g	Twice daily	PCOS supplement
Vitamin D3	60,000 IU	Once weekly	12-week course
Calcium + Magnesium	500mg	Once daily	Bone support
Sertraline	50mg	Once daily (morning)	Continue — psychiatry prescribed

FOLLOW-UP PLAN

- Endocrinology OPD in 6 weeks — repeat TSH, Free T4. Dose adjustment if needed.
- Psychiatry OPD in 4 weeks — depression review, Sertraline dose assessment
- Gynaecology OPD in 8 weeks — PCOS management, menstrual regulation
- Repeat HbA1c, Fasting sugar, Vitamin D in 3 months
- Weight target: Lose 5–7% body weight (4–6 kg) over 6 months — significantly improves PCOS and insulin resistance

AVOID: Codeine, tramadol, and all opioid analgesics — documented intolerance. Use Paracetamol for pain.